

D026

Quality Outcomes in a Culture of Value-Based Nursing Care

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

Two diabetes-education programs are compared. Program A achieves an outcome score of 80 at a cost of \$8,000 per patient. Program B achieves 90 at \$12,000 per patient. Program B is run by a nationally known clinic. Which has higher value?

- A. Program B, because its outcome is higher.
- B. Program A ($80 / 8,000 = 0.0100$ vs. $90 / 12,000 = 0.0075$).
- C. Program B, because of the clinic's reputation.
- D. They are equal, because B costs 50% more and scores 12.5% higher.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A unit keeps its outcomes steady while cutting cost per patient by 20%. By how much does value change?

- A. It rises 20%.
- B. No change, because outcomes did not change.
- C. It falls 20%.
- D. It rises 25%.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A hospital with a new parking garage tracks the percentage of surgical patients who receive antibiotics within 60 minutes before incision. The surgery department has 14 operating rooms. What type of measure is this?

- A. Process measure.
- B. Balancing measure.
- C. Structure measure, because it involves operating rooms.
- D. Outcome measure.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

Which is an outcome measure?

- A.** Nurse-to-patient ratio.
- B.** Number of staff trained on catheter care.
- C.** Percentage of catheters removed by day 2.
- D.** Rate of catheter-associated urinary tract infections.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A nurse manager wants to try bedside shift report. The unit has 32 beds and just won a recognition award. Staff are unsure it will work. Using PDSA, what is the best first step?

- A.** Test bedside report with one nurse pair on one shift, then study the results.
- B.** Require bedside report on all 32 beds starting Monday.
- C.** Never change report until all staff agree.
- D.** Skip testing, since an award-winning unit will succeed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

An outpatient infusion center finds patients wait 90 minutes because of repeated trips to the pharmacy, duplicate paperwork, and idle chairs. The center recently moved to a new floor. Which method best fits?

- A.** Relocate the center again.
- B.** Six Sigma, to reduce statistical variation.
- C.** Lean, to remove waste and non-value-added steps.
- D.** Root cause analysis of a sentinel event.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A hospital with a large nursing residency program plans to launch a new smart-pump drug library next month. No patient has been harmed. Leadership wants to find how the process could fail. Which tool fits?

- A.** A patient satisfaction survey.
- B.** Root cause analysis (RCA).
- C.** Additional training for residents only.
- D.** Failure mode and effects analysis (FMEA).

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A patient received a tenfold insulin overdose last week. Which tool should the team use now?

- A.** Root cause analysis (RCA).
- B.** Discipline the nurse, which closes the event.
- C.** FMEA.
- D.** PDSA cycle only.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A nurse with a spotless 12-year record skips scanning a patient's wristband because the scanner is slow and many peers do the same. No harm results. The unit recently switched to new scanners. Under just culture, what is the best response?

- A.** Terminate the nurse for a safety violation.
- B.** Coach the nurse and fix the slow scanner that drives the shortcut.
- C.** Take no action because her record is spotless.
- D.** Console the nurse for an unintentional slip.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A health system joins a shared-savings ACO contract. It employs 400 nurses and recently opened a new clinic. Under this model, which action most directly improves its financial result while meeting quality targets?

- A.** Increase the number of billed visits.
- B.** Open more new clinics.
- C.** Care coordination that prevents avoidable ED visits and admissions.
- D.** Negotiate a single price for each knee replacement.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A hospital had 500 discharges of heart failure patients last quarter; 60 were readmitted within 30 days. The hospital had 2,000 total discharges that quarter and a new cardiology wing. What is the heart failure 30-day readmission rate?

- A.** 60
- B.** 8.3%
- C.** 12%
- D.** 3%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

12. [TRANSFER — reversed stem / different variable]

A hospital expects 2,000 total discharges next quarter, 500 of them heart failure patients. It wants a heart failure 30-day readmission rate of 10% or lower. What is the most heart failure readmissions it can have and still meet the goal?

- A.** 10
- B.** 50
- C.** 5,000
- D.** 200

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

Two diabetes-education programs are compared. Program A achieves an outcome score of 80 at a cost of \$8,000 per patient. Program B achieves 90 at \$12,000 per patient. ~~Program B is run by a nationally known clinic.~~ Which has higher value?

Target word: higher value **Decides:** Outcome per dollar.

A	Dropped element	Ignores cost in the value ratio.
B	Correct	A delivers more outcome per dollar.
C	Camouflage	Reputation is not part of value.
D	Inverted operation	The percentages show B's outcome rises less than its cost, so value falls.

2. Answer D Concept 1 • transfer

A unit keeps its outcomes steady while cutting cost per patient by 20%. By how much does value change?

Target word: solve for change in value **Decides:** Value = outcome / (0.8 × cost).

A	Inverted operation	Assumes value moves by the same percent as cost.
B	Dropped element	Drops cost from the ratio.
C	Confusion set	Lower cost with the same outcome raises value.
D	Correct	$1 / 0.8 = 1.25$.

3. Answer A Concept 2

A hospital ~~with a new parking garage~~ tracks the percentage of surgical patients who receive antibiotics within 60 minutes before incision. ~~The surgery department has 14 operating rooms.~~ What type of measure is this?

Target word: type of measure **Decides:** Whether a care step was done.

A	Correct	Tracks whether a care action was delivered.
B	Adjacent category	Balancing measures watch for unintended effects elsewhere.
C	Camouflage	Room count is structure, but the measure is a care step.
D	Confusion set	An outcome would be the infection rate, not the antibiotic timing.

4. Answer D Concept 2 • transfer

Which is an outcome measure?

Target word: identify an outcome **Decides:** Result for the patient.

A	Adjacent category	A resource: structure.
B	Inverted operation	An input count, not a patient result.
C	Confusion set	A care step: process.
D	Correct	A patient result is an outcome.

5. Answer A Concept 3

A nurse manager wants to try bedside shift report. ~~The unit has 32 beds and just won a recognition award.~~ Staff are unsure it will work. Using PDSA, what is the best first step?

Target word: first PDSA step **Decides:** Test small, then study.

A	Correct	PDSA starts with a small test and data.
B	Inverted operation	Scales before testing.
C	Absolute language	Unanimity is not required for a small test.
D	Camouflage	The award does not predict this change's effect.

6. Answer C Concept 4

An outpatient infusion center finds patients wait 90 minutes because of repeated trips to the pharmacy, duplicate paperwork, and idle chairs. ~~The center recently moved to a new floor.~~ Which method best fits?

Target word: which method **Decides:** Waste and flow problems.

A	Camouflage	The move is not the cause of the waste.
B	Confusion set	Six Sigma targets variation and defects, not flow waste.
C	Correct	Waiting, extra motion, and duplication are classic Lean wastes.
D	Adjacent category	No single harm event is described.

7. Answer D Concept 5

A hospital ~~with a large nursing residency program~~ plans to launch a new smart-pump drug library next month. No patient has been harmed. Leadership wants to find how the process could fail. Which tool fits?

Target word: which tool **Decides:** Prospective analysis of a new process.

A	Adjacent category	Does not analyze process failure.
B	Inverted operation	RCA looks back at an event that already happened.
C	Camouflage	The residency program does not decide the tool.
D	Correct	Looks ahead to find failures before harm.

8. Answer A Concept 5 • transfer

A patient received a tenfold insulin overdose last week. Which tool should the team use now?

Target word: tool after an event **Decides:** Event already happened.

A	Correct	Retrospective analysis of an event that occurred.
B	Absolute language	Blame does not find system causes.
C	Inverted operation	FMEA is prospective, before failure.
D	Adjacent category	PDSA tests changes; it does not find the root cause first.

9. Answer B Concept 6

A nurse ~~with a spotless 12-year record~~ skips scanning a patient's wristband because the scanner is slow and many peers do the same. No harm results. ~~The unit recently switched to new scanners.~~ Under just culture, what is the best response?

Target word: just culture response **Decides:** At-risk behavior: a shortcut where risk was not recognized.

A	Absolute language	Punitive response fits reckless behavior, not a common shortcut.
B	Correct	At-risk behavior calls for coaching and system repair.
C	Camouflage	Her record does not change the behavior's category.
D	Confusion set	A deliberate shortcut is at-risk behavior, not human error.

10. Answer C Concept 7

A health system joins a shared-savings ACO contract. ~~It employs 400 nurses and recently opened a new clinic.~~ Under this model, which action most directly improves its financial result while meeting quality targets?

Target word: who gains under shared savings **Decides:** Lower total cost of care for the population.

A	Inverted operation	Rewards volume, which fits fee-for-service.
B	Camouflage	Expansion adds cost; the new clinic is not the lever.
C	Correct	Shared savings rewards reducing total cost with quality met.
D	Confusion set	That describes a bundled payment.

11. Answer C Concept 8

A hospital had 500 discharges of heart failure patients last quarter; 60 were readmitted within 30 days. ~~The hospital had 2,000 total discharges that quarter and a new cardiology wing.~~ What is the heart failure 30-day readmission rate?

Target word: readmission rate **Decides:** Readmits / heart failure discharges.

A	Dropped element	The count without a denominator.
B	Inverted operation	Divides 500 by 60 and misplaces the decimal.
C	Correct	$60 / 500 = 0.12$.
D	Wrong stem number	Uses all 2,000 discharges as the denominator.

12. Answer B Concept 8 • transfer

A hospital expects 2,000 total discharges next quarter, 500 of them heart failure patients. It wants a heart failure 30-day readmission rate of 10% or lower. What is the most heart failure readmissions it can have and still meet the goal?

Target word: solve for count **Decides:** Events = rate × denominator.

A	Wrong stem number	The target percent read as a count.
B	Correct	$10\% \times 500 = 50$.
C	Inverted operation	Divides 500 by 0.10 instead of multiplying.
D	Adjacent category	Applies 10% to all 2,000 hospital discharges rather than the at-risk group.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	9	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	7	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Value = outcomes / cost	Value in health care is the health outcomes achieved per dollar spent; it rises when outcomes improve or cost falls with outcomes held steady.	Treating the cheapest or the highest-quality option alone as highest value.	Two options with both an outcome measure and a cost.
2	Donabedian: structure, process, outcome	Structure = resources and setting; process = what care is done; outcome = the result for the patient.	Calling a care step, like a checklist done, an outcome.	"What type of measure is this?"
3	PDSA small tests of change	Plan-Do-Study-Act tests a change on a small scale, studies the data, then adopts, adapts, or abandons it.	Rolling a change out unit-wide before a small test.	A new idea with unknown effects.
4	Lean vs. Six Sigma	Lean removes waste and non-value-added steps to improve flow; Six Sigma reduces variation and defects using data (DMAIC).	Using the two names as the same method.	Delays and wasted steps vs. inconsistent results or error rates.
5	RCA vs. FMEA	Root cause analysis looks back at an event that already happened; FMEA looks ahead to find how a process could fail before harm occurs.	Using RCA for a process that has not yet caused harm.	Has an event happened yet, or is a new process being launched?
6	Just culture	Console human error, coach at-risk behavior, and hold reckless behavior accountable; focus on systems, not blame.	Punishing every error, or never holding anyone accountable.	A staff member's choice described as a slip, a shortcut, or a conscious disregard of risk.
7	Payment models and risk	Fee-for-service pays per service (volume); bundled payments pay one price for an episode; shared savings (ACO) rewards lower total cost with quality met.	Thinking fee-for-service rewards value.	Who gains when services are reduced or avoided.
8	Rates need the right denominator	A rate = events / the population that could have had the event, over a defined period.	Using the wrong denominator, such as all patients instead of those at risk.	A count of events next to more than one possible total.

Course scope (public description): Quality Outcomes in a Culture of Value-Based Nursing Care incorporates current standards of quality and safety within the context of value-based care. In a value-based healthcare system, the benefits are derived from measuring health outcomes against the cost of delivering the outcomes. These benefits are then extended to patients, providers, payers, suppliers, and society as a whole. This course introduces new healthcare delivery models, which stress a team-oriented approach to patient care and sharing of patient data so that care is coordinated, and outcomes can be measured easily. Emphasis is placed on performance and quality improvement methods that underlie value-based nursing care. The nurse in advanced practice today must exemplify the standards of quality and safety and be prepared to lead the delivery of value-based patient-centered care.